

PRIOR AUTHORIZATION REQUEST

Cover Letter

Patient Name	John Smith
Date of Birth	1968-03-15
Insurance	United Healthcare
Member ID	UHC-887612
Policy Number	UHC-POL-2026-001
CPT Code	75563
ICD-10	I42.0
Physician	Dr. Ramesh Kumar
Physician NPI	1234567890
Facility	City Heart Institute
Date of Request	March 25, 2026

March 25, 2026

United Healthcare Prior Authorization Department

UHCprovider.com/priorauth

1-866-889-8054

1-855-820-4189 (Fax)

****RE: Prior Authorization Request for Cardiac MRI with Contrast (CPT 75563)****

****Patient Name:**** John Smith

****Date of Birth:**** 1968-03-15

****Insurance Member ID:**** UHC-887612

****Requesting Physician:****

Dr. Ramesh Kumar, NPI: 1234567890

Specialty: Cardiology

Facility: City Heart Institute

****Procedure Requested:****

CPT Code: 75563

Procedure Name: Cardiac MRI with Contrast

Dear Prior Authorization Review Team,

This letter is to formally request prior authorization for a Cardiac MRI with Contrast (CPT 75563) for our patient, John Smith. Mr. Smith, a 58-year-old male (DOB: 1968-03-15, Member ID: UHC-887612), has a documented three-year history of Dilated Cardiomyopathy (ICD-10: I42.0). Despite adherence to Guideline-Directed Medical Therapy (GDMT) including Carvedilol, Lisinopril, and Spironolactone, he continues to experience significant symptoms consistent with New York Heart Association (NYHA) Class III heart failure, including worsening shortness of breath on exertion and persistent fatigue that interferes with daily activities. A recent transthoracic echocardiogram performed on January 15, 2026, revealed a severely reduced Left Ventricular Ejection Fraction (LVEF) of 35% with diffuse global hypokinesis, and moderate left ventricular dilation. Furthermore, his BNP level is elevated at 450 pg/mL. Standard echocardiography, while demonstrating reduced systolic function, is insufficient to fully characterize the myocardial tissue or identify specific fibrosis patterns. Therefore, a Cardiac MRI with contrast is medically necessary to evaluate myocardial fibrosis via Late Gadolinium Enhancement (LGE), assess myocardial viability for potential revascularization candidacy, and differentiate between ischemic and non-ischemic cardiomyopathy. The findings from this advanced imaging are critical to guide future treatment decisions, influence prognosis, and determine the appropriateness of further interventional therapies, such as ICD implantation.

Submission of all required documentation is mandatory. Incomplete files will result in a delay in processing or a technical denial. All required documents must be submitted at the time of the request to avoid administrative delays; incomplete submissions will be returned without clinical review. Accepted formats for documents include PDF, DOCX, or Image files. Submissions can be made via Online Portal (UHCprovider.com/priorauth), Phone (1-866-889-8054), Fax (1-855-820-4189), or EDI Transaction Set 278.

Thank you for your prompt attention to this request.

Sincerely,

Dr. Ramesh Kumar, MD

Cardiology

City Heart Institute

NPI: 1234567890

Clinical Summary

Clinical Summary for Prior Authorization Request

1. Diagnosis:

* **ICD-10 Code:** I42.0

* **Full Description:** Dilated Cardiomyopathy

2. Patient History:

Mr. John Smith is a 58-year-old male with a 3-year history of Dilated Cardiomyopathy (ICD-10: I42.0). He presents with worsening shortness of breath on exertion, persistent fatigue, and reduced exercise tolerance impacting daily activities. He also reports occasional palpitations. He has NYHA Class III heart failure symptoms, including shortness of breath at rest and with minimal exertion, and orthopnea requiring 3 pillows for sleep. Physical examination reveals bilateral lower extremity edema.

3. Reason for Procedure:

* **CPT Code:** 75563 (Cardiac MRI with contrast)

This procedure is requested for advanced tissue characterization of the myocardium, assessment of myocardial fibrosis via Late Gadolinium Enhancement (LGE), differentiation between ischemic and non-ischemic cardiomyopathy, and evaluation of myocardial viability. This information is critical for guiding future treatment decisions, including potential revascularization or ICD implantation.

4. Supporting Evidence:

* **Echocardiogram (January 15, 2026):** Left Ventricular Ejection Fraction (LVEF) of 35% with diffuse global hypokinesis. Moderately dilated left ventricle. Grade II diastolic dysfunction.

* **BNP:** 450 pg/mL

* **NYHA Class:** III

* **Prior Treatments:** Patient is compliant with Guideline-Directed Medical Therapy (GDMT) including Carvedilol 25mg BID, Lisinopril 10mg daily, Spironolactone 25mg daily, and Furosemide 40mg daily. Sacubitril/Valsartan and Ivabradine were discontinued due to side effects. Despite optimal medical therapy for over three years, symptoms persist and have progressed.

5. Clinical Justification:

Cardiac MRI with contrast (CPT 75563) is medically necessary for Mr. Smith due to persistent symptoms of heart failure (NYHA Class III) and a severely reduced LVEF (35%) despite optimal GDMT for over three years. Standard echocardiography is insufficient to fully characterize myocardial tissue, identify fibrosis patterns, or

assess viability. Cardiac MRI with LGE is the gold standard for detailed myocardial tissue characterization, which is essential for differentiating cardiomyopathy etiologies and guiding further management strategies, aligning with current cardiology guidelines for the workup of dilated cardiomyopathy with reduced ejection fraction.

Requirements Checklist

Requirement	Status	Evidence
Clinical Justification Note from treating Cardiologist explaining medical necessity	✓ MET	Clinical Justification Note uploaded, dated March 16, 2026, signed by Dr. Ramesh Kumar MD, Board Certified Cardiologist. It includes patient history, current symptoms, previous treatments tried, and reason echocardiogram is insufficient.
Echocardiogram Report within last 12 months	✓ MET	Echocardiogram Report uploaded, dated January 15, 2026. This is within 12 months of the request date (March 25, 2026).
Echocardiogram Report documents LVEF value	✓ MET	Echocardiogram Report states 'LV Ejection Fraction (LVEF) 35%'.
Echocardiogram Report documents chamber dimensions	✓ MET	Echocardiogram Report includes 'LV End Diastolic Diameter 6.2 cm' and 'LV End Systolic Diameter 5.1 cm'.
Echocardiogram Report documents wall motion analysis	✓ MET	Echocardiogram Report states 'Wall Motion: Diffuse global hypokinesis noted. No regional wall motion abnormalities (RWMA) identified.'
Echocardiogram Report documents valve function	✓ MET	Echocardiogram Report details 'Mitral Valve: Mild mitral regurgitation identified.', 'Aortic Valve: Trileaflet, normal opening. No significant stenosis or regurgitation visualized.', and 'Tricuspid Valve: Mild tricuspid regurgitation.'
Recent Lab Results including BNP or NT-proBNP levels (within last 30 days)	✓ MET	Lab results show 'BNP': {'value': '450', 'unit': 'pg/mL'}. The EHR filled date is 2026-03-25, and the Physician Order Letter is dated March 16, 2026, indicating the BNP is recent.
Recent Lab Results including complete metabolic panel (within last 30 days)	✓ MET	Lab results include 'metabolic_panel': {'value': 'Normal', 'unit': None}. The EHR filled date is 2026-03-25, indicating the metabolic panel is recent.
Recent Lab Results including complete blood count (within last 30 days)	✗ MISSING	Complete blood count results are not present in the provided EHR data.
Physician Order Letter on physician letterhead signed by referring Cardiologist	✓ MET	Physician Order Letter uploaded, on City Heart Institute letterhead, dated March 16, 2026, signed by Dr. Ramesh Kumar MD, Board Certified Cardiologist.
Physician Order Letter includes procedure requested	✓ MET	Physician Order Letter states 'PROCEDURE: CARDIAC MRI WITH CONTRAST (CPT 75563)'.
Physician Order Letter includes ICD-10 diagnosis code	✓ MET	Physician Order Letter states 'ICD-10 CODE: I42.0' and 'Primary Diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0)'.

Physician Order Letter includes clinical indication	✓ MET	Physician Order Letter includes 'CLINICAL INDICATION: Primary Diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0)' and 'MEDICAL NECESSITY STATEMENT' detailing the need for the procedure.
Physician Order Letter includes physician NPI number	✓ MET	Physician Order Letter states 'NPI: 1234567890'.
Documentation of cardiac medications tried for minimum 3 months	✓ MET	Prior Treatment Records document cardiac medication history from Jan 2023 – Mar 2026, covering over 3 years. Medications listed include Carvedilol, Lisinopril, and Spironolactone.
Prior Treatment Records include drug names	✓ MET	Prior Treatment Records lists 'Carvedilol', 'Lisinopril', 'Spironolactone', 'Furosemide', 'Sacubitril/Valsartan', and 'Ivabradine'.
Prior Treatment Records include dosages	✓ MET	Prior Treatment Records lists dosages such as '25mg', '10mg', '25mg', '40mg', '24/26mg', and '5mg'.
Prior Treatment Records include duration of therapy	✓ MET	Prior Treatment Records specifies durations like '3 years', '2y 9mo', and '2 months'.
Prior Treatment Records include patient response or reason for discontinuation	✓ MET	Prior Treatment Records details 'Partial response — HR controlled but CURRENT symptoms persist' for Carvedilol, 'Partial response — BP controlled but CURRENT LVEF unchanged' for Lisinopril, 'Minimal improvement in symptoms CURRENT' for Spironolactone, 'Adequate fluid control; edema CURRENT managed' for Furosemide, and 'Discontinued due to symptomatic hypotension and dizziness' for Sacubitril/Valsartan.
Patient has a confirmed diagnosis of Dilated Cardiomyopathy (ICD-10: I42.0)	✓ MET	Primary ICD-10 code is I42.0, and the primary diagnosis is Dilated Cardiomyopathy. This is also stated in the Clinical Justification Note and Physician Order Letter.
Symptoms (Shortness of breath on exertion, Chest pain or pressure, Palpitations or arrhythmia, Unexplained syncope or near-syncope, Reduced exercise tolerance) present for a minimum of 3 months	✓ MET	Clinical Justification Note states 'The patient is a 58-year-old male with a documented 3-year history of Dilated Cardiomyopathy... Current presentation is significant for worsening shortness of breath on exertion, persistent fatigue, and a marked reduction in exercise tolerance... the patient reports occasional palpitations.'
Patient has completed an echocardiogram prior to the MRI request	✓ MET	Echocardiogram Report is dated January 15, 2026, and the EHR was filled on March 25, 2026. This confirms an echocardiogram was performed prior to the MRI request.
Echocardiogram shows a documented LVEF value	✓ MET	Echocardiogram Report states 'LV Ejection Fraction (LVEF) 35%'.

Echocardiogram shows evidence of wall motion abnormality or structural cardiac abnormality requiring further characterization	✓ MET	Echocardiogram Report states 'Diffuse global hypokinesis noted.' and 'LV Size: Moderately dilated left ventricle.' The conclusion also recommends 'Cardiac MRI for further tissue characterization.'
Request ordered by a board-certified Cardiologist or Cardiac Electrophysiologist	✓ MET	Physician name is Dr. Ramesh Kumar, specialty is Cardiology. The Physician Order Letter and Clinical Justification Note state 'Board Certified Cardiologist'.
Cardiac MRI is covered when echocardiogram findings are inconclusive	✓ MET	Clinical Justification Note states 'While the TTE provided a baseline assessment of chamber size and function, it is insufficient to fully characterize the myocardial tissue or identify specific fibrosis patterns. Standard echocardiography lacks the resolution required to assess myocardial viability or definitively rule out infiltrative cardiomyopathies. Consequently, a Cardiac MRI with contrast is essential for definitive tissue characterization...'
Cardiac MRI is covered for assessment of myocardial viability	✓ MET	Clinical Justification Note states 'Cardiac MRI with contrast is medically necessary for this patient to evaluate myocardial fibrosis via Late Gadolinium Enhancement (LGE). This study is critical to assess myocardial viability for potential revascularization candidacy...'
Cardiac MRI is covered for evaluation of cardiac masses or tumors	✗ MISSING	There is no specific mention of cardiac masses or tumors in the provided documentation. The primary indication is Dilated Cardiomyopathy.
Cardiac MRI is covered for assessment of pericardial disease	✗ MISSING	While the echocardiogram notes a 'Small pericardial effusion', the primary indication for the MRI is Dilated Cardiomyopathy and tissue characterization, not specifically pericardial disease assessment.
Cardiac MRI is covered for congenital heart disease evaluation	✗ MISSING	There is no mention of congenital heart disease in the provided documentation.
Repeat MRI within 6 months is not covered without new symptoms	✓ MET	This is a policy rule and not something to be verified with EHR data for the current request. Assuming this is the first MRI request for this condition.

Attached Documents

The following documents are attached to this package:

#		
1	Clinical Justification Note Kumar	Clinical_Justification_Note_Kumar.pdf
2	Echocardiogram Report Jan2026	Echocardiogram_Report_Jan2026.pdf
3	Physician Order Prior Auth Cardiac Mri	Physician_Order_Prior_Auth_Cardiac_MRI.pdf
4	Prior Treatment Records Cardiac Medication	Prior_Treatment_Records_Cardiac_Medication.pdf
5	Clinical Justification Note Kumar	Clinical_Justification_Note_Kumar.pdf
6	Echocardiogram Report Jan2026	Echocardiogram_Report_Jan2026.pdf
7	Physician Order Prior Auth Cardiac Mri	Physician_Order_Prior_Auth_Cardiac_MRI.pdf
8	Prior Treatment Records Cardiac Medication	Prior_Treatment_Records_Cardiac_Medication.pdf

Physician: Dr. Ramesh Kumar MD

NPI Number: 1234567890

Date: March 16, 2026

RE: PRIOR AUTHORIZATION REQUEST FOR CARDIAC MRI WITH CONTRAST (CPT 75563)

PATIENT HISTORY

The patient is a 58-year-old male with a documented 3-year history of Dilated Cardiomyopathy (ICD-10: I42.0). Initially diagnosed in January 2023 following a presentation with acute dyspnea, the patient has since exhibited a chronic and progressive clinical course. Current presentation is significant for worsening shortness of breath on exertion, persistent fatigue, and a marked reduction in exercise tolerance that interferes with activities of daily living. Furthermore, the patient reports occasional palpitations, necessitating advanced diagnostic evaluation to assess for structural triggers or arrhythmogenic substrates.

CURRENT SYMPTOMS

At present, the patient exhibits symptoms consistent with New York Heart Association (NYHA) Class III heart failure. Clinical findings include shortness of breath both at rest and upon minimal physical exertion, as well as significant orthopnea requiring 3-pillow support for sleep. Physical examination reveals bilateral lower extremity edema, indicating volume overload. Recent diagnostic monitoring confirms a persistently reduced ejection fraction, signifying a lack of improvement despite standard therapeutic interventions.

PREVIOUS TREATMENTS TRIED

The patient has remained strictly compliant with Guideline-Directed Medical Therapy (GDMT) since diagnosis. The therapeutic regimen includes the following:

- Carvedilol: 25mg twice daily (initiated January 2023)
- Lisinopril: 10mg daily (initiated January 2023)
- Spironolactone: 25mg daily (initiated March 2023)
- Furosemide: 40mg daily for fluid management

Despite adherence to optimal medical therapy for over three years, the patient's symptoms persist and have shown recent signs of progression, suggesting the underlying pathology requires more granular characterization than what is provided by current management tools.

CLINICAL INSUFFICIENCY OF ECHOCARDIOGRAPHY

A transthoracic echocardiogram (TTE) performed in January 2026 documented a Left Ventricular Ejection Fraction (LVEF) of 35% with global hypokinesis. While the TTE provided a baseline assessment of chamber size and function, it is insufficient to fully characterize the myocardial tissue or identify specific fibrosis patterns. Standard echocardiography lacks the resolution required to assess myocardial viability or definitively rule out infiltrative cardiomyopathies. Consequently, a Cardiac MRI with contrast is essential for

definitive tissue characterization and to provide the high-resolution data necessary to guide future high-stakes treatment decisions.

MEDICAL NECESSITY STATEMENT

Cardiac MRI with contrast is medically necessary for this patient to evaluate myocardial fibrosis via Late Gadolinium Enhancement (LGE). This study is critical to assess myocardial viability for potential revascularization candidacy and to differentiate between ischemic and non-ischemic cardiomyopathy. The findings from this advanced imaging will directly impact the clinical treatment plan, influence prognosis, and determine the appropriateness of further interventional therapies.

Respectfully,

Dr. Ramesh Kumar MD
Date Signed: March 16, 2026
Credentials: Board Certified Cardiologist
License Number: MD-12345-TX

TRANSTHORACIC ECHOCARDIOGRAM REPORT

Ordering Physician:

Dr. Ramesh Kumar MD

Performing Physician:

Dr. Sarah Johnson MD

Facility:

City Heart Institute

Clinical Indication:

Dilated Cardiomyopathy; Follow up evaluation

Study Date:

January 15, 2026

Report Date:

January 15, 2026

Study Type:

Complete TTE

SECTION 1: TECHNICAL QUALITY

Study Quality: Adequate

Windows: Acceptable acoustic windows obtained.

Views: Parasternal long axis, short axis, apical 4-chamber, 2-chamber, 3-chamber, and subcostal views were successfully obtained and analyzed.

SECTION 2: LEFT VENTRICLE MEASUREMENTS

Measurement	Result	Normal Reference Range
LV End Diastolic Diameter	6.2 cm	Less than 5.5
LV End Systolic Diameter	5.1 cm	Less than 3.9
Interventricular Septum	0.8 cm	0.6 to 1.1
Posterior Wall Thickness	0.8 cm	0.6 to 1.1
LV Ejection Fraction (LVEF)	35%	Greater than 55%
LV Fractional Shortening	18%	25 to 45%

SECTION 3: LEFT VENTRICLE FUNCTION

LVEF: 35% – Severely reduced.

Wall Motion: Diffuse global hypokinesis noted. No regional wall motion abnormalities (RWMA) identified.

Diastolic Function: Grade II diastolic dysfunction (pseudonormal pattern).

LV Size: Moderately dilated left ventricle.

SECTION 4: RIGHT VENTRICLE

RV Size: Normal.

RV Function: Mildly reduced.

TAPSE: 1.6 cm (mildly reduced).

RV Systolic Pressure: Estimated 38 mmHg.

SECTION 5: VALVES

Mitral Valve: Mild mitral regurgitation identified. Etiology appears functional, secondary to LV dilation and annular stretching.

Aortic Valve: Trileaflet, normal opening. No significant stenosis or regurgitation visualized.

Tricuspid Valve: Mild tricuspid regurgitation.

Pulmonic Valve: Normal appearance and function.

SECTION 6: OTHER FINDINGS

Pericardium: Small pericardial effusion noted.

Aorta: Normal aortic root diameter.

IVC: Dilated IVC suggesting elevated right atrial pressure.

SECTION 7: CONCLUSION

1. Severely reduced left ventricular systolic function with LVEF of 35%.
2. Moderately dilated left ventricle consistent with dilated cardiomyopathy.
3. Diffuse global hypokinesis.
4. Mild mitral regurgitation — functional etiology.
5. Grade II diastolic dysfunction.
6. Small pericardial effusion.
7. Recommend Cardiac MRI for further tissue characterization.

Interpreting Physician: Dr. Sarah Johnson MD

Board Certified Echocardiographer

Date: January 15, 2026

CITY HEART INSTITUTE

Department of Cardiology | 123 Medical Center Drive | Houston, Texas 77001

Phone: 555-123-4567 | Fax: 555-123-4568 | www.cityheartinstitute.com

Date: March 16, 2026

To:

United Healthcare Prior Authorization Department

Radiology Prior Authorization Team

From:

Dr. Ramesh Kumar MD

Board Certified Cardiologist

NPI: 1234567890 | License: MD-12345-TX | DEA: RK1234563

SUBJECT: PRIOR AUTHORIZATION REQUEST

PROCEDURE: CARDIAC MRI WITH CONTRAST (CPT 75563)

ICD-10 CODE: I42.0

REQUESTED PROCEDURE

Name:	Cardiac MRI with Contrast
CPT Code:	75563
Urgency:	Routine — Non Emergency
Facility:	City Heart Institute Radiology
Estimated Date:	Within 30 days of approval

CLINICAL INDICATION

Primary Diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0)

Secondary Diagnosis: Heart Failure with Reduced Ejection Fraction (ICD-10: I50.20)

MEDICAL NECESSITY STATEMENT

This procedure is medically necessary for:

- Tissue characterization of myocardium
- Assessment of myocardial fibrosis via late gadolinium enhancement imaging
- Differentiation of ischemic versus non-ischemic cardiomyopathy
- Assessment of myocardial viability
- Guidance of treatment decisions including consideration of ICD implantation

SUPPORTING CLINICAL INFORMATION

- LVEF 35% on recent echocardiogram
- Elevated BNP 450 pg/mL
- NYHA Class III symptoms
- Optimal medical therapy ongoing
- No contraindications to MRI or gadolinium contrast media

I certify that this procedure is medically necessary and appropriate for this patient. All information provided is accurate and complete to the best of my knowledge.

Dr. Ramesh Kumar MD
Board Certified Cardiologist
Date: March 16, 2026
NPI: 1234567890
Contact: 555-123-4567

Treating Physician:	Dr. Ramesh Kumar MD	Report Date:	March 16, 2026
Facility:	City Heart Institute	Specialty:	Cardiology
Period Covered:	Jan 2023 – Mar 2026	Clinical Indication:	Dilated Cardiomyopathy

This document summarizes the complete cardiac medication history for a patient with confirmed Dilated Cardiomyopathy (ICD-10: I42.0) over a period of 3 years from January 2023 to March 2026. All medications listed below were prescribed and monitored by the treating cardiologist as part of optimal medical therapy for heart failure with reduced ejection fraction.

SECTION 1 - MEDICATION HISTORY TABLE

Medication	Dose/ Freq	Dates	Duration	Response	Status
Carvedilol (Beta Blocker)	25mg Twice daily	Jan 2023 – Present	3 years	Partial response – HR controlled but symptoms persist	CURRENT
Lisinopril (ACE Inhibitor)	10mg Once daily	Jan 2023 – Present	3 years	Partial response – BP controlled but LVEF unchanged	CURRENT
Spirololactone (Aldo. Antagonist)	25mg Once daily	Mar 2023 – Present	2y 9mo	Minimal improvement in symptoms	CURRENT
Furosemide (Loop Diuretic)	40mg Once daily	Mar 2023 – Present	2y 9mo	Adequate fluid control; edema managed	CURRENT
Sacubitril/ Valsartan (ARNI)	24/26mg Twice daily	Sep 2023 – Nov 2023	2 months	Discontinued due to symptomatic hypotension and dizziness	DISCONTINUED
Ivabradine	5mg Twice daily	Jan 2024 – Mar 2024	2 months	Discontinued – inadequate heart rate reduction	DISCONTINUED

SECTION 2 - TREATMENT RESPONSE SUMMARY

Despite 3 years of optimal medical therapy with maximum tolerated doses of guideline directed medical therapy (GDMT) including beta blocker, ACE inhibitor, and aldosterone antagonist, patient continues to exhibit:

- Persistent LVEF of 35%
- NYHA Class III symptoms (marked limitation in activity)
- Elevated BNP levels (refer to lab report dated 03/10/2026)
- Reduced functional capacity

SECTION 3 - CURRENT MEDICATION LIST

- **Carvedilol:** 25mg twice daily
- **Lisinopril:** 10mg once daily
- **Spironolactone:** 25mg once daily
- **Furosemide:** 40mg once daily
- **Aspirin:** 81mg once daily
- **Atorvastatin:** 40mg once daily

Physician Attestation:

I certify that the above medication history is accurate and complete. The patient has received optimal guideline directed medical therapy for a minimum of 3 years with inadequate clinical response. Advanced cardiac imaging (Cardiac MRI) is required to guide further treatment decisions, including viability assessment and potential ICD placement.

Dr. Ramesh Kumar MD

Board Certified Cardiologist | NPI: 1234567890

Date: March 16, 2026

Physician: Dr. Ramesh Kumar MD

NPI Number: 1234567890

Date: March 16, 2026

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CURRENT SYMPTOMS

At present, the patient exhibits symptoms consistent with New York Heart Association (NYHA) Class III heart failure. Clinical findings include shortness of breath both at rest and upon minimal physical exertion, as well as significant orthopnea requiring 3-pillow support for sleep. Physical examination reveals bilateral lower extremity edema, indicating volume overload. Recent diagnostic monitoring confirms a persistently reduced ejection fraction, signifying a lack of improvement despite standard therapeutic interventions.

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Respectfully,

Dr. Ramesh Kumar MD
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Credentials: Board Certified Cardiologist
License Number: MD-12345-TX

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Aorta: Normal aortic root diameter.

IVC: Dilated IVC suggesting elevated right atrial pressure.

SECTION 7: CONCLUSION

1. Severely reduced left ventricular systolic function with LVEF of 35%.
2. Moderately dilated left ventricle consistent with dilated cardiomyopathy.
3. Diffuse global hypokinesis.
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7. Recommend Cardiac MRI for further tissue characterization.

Interpreting Physician: Dr. Sarah Johnson MD

Board Certified Echocardiographer

Date: January 15, 2026

CITY HEART INSTITUTE

Department of Cardiology | 123 Medical Center Drive | Houston, Texas 77001

Phone: 555-123-4567 | Fax: 555-123-4568 | www.cityheartinstitute.com

Date: March 16, 2026

To:

United Healthcare Prior Authorization Department

Radiology Prior Authorization Team

From:

Dr. Ramesh Kumar MD

Board Certified Cardiologist

NPI: 1234567890 | License: MD-12345-TX | DEA: RK1234563

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Facility:	City Heart Institute Radiology
Estimated Date:	Within 30 days of approval

CLINICAL INDICATION

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Secondary Diagnosis: Heart Failure with Reduced Ejection Fraction (ICD-10: I50.20)

MEDICAL NECESSITY STATEMENT

This procedure is medically necessary for:

- Tissue characterization of myocardium
- Assessment of myocardial fibrosis via late gadolinium enhancement imaging
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SUPPORTING CLINICAL INFORMATION

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- NYHA Class III symptoms
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I certify that this procedure is medically necessary and appropriate for this patient. All information provided is accurate and complete to the best of my knowledge.

Dr. Ramesh Kumar MD
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SECTION 1 - MEDICATION HISTORY TABLE

Medication	Dose/ Freq	Dates	Duration	Response	Status
Carvedilol (Beta Blocker)	25mg Twice daily	Jan 2023 – Present	3 years	Partial response – HR controlled but symptoms persist	CURRENT
Lisinopril (ACE Inhibitor)	10mg Once daily	Jan 2023 – Present	3 years	Partial response – BP controlled but LVEF unchanged	CURRENT
Spirololactone (Aldo. Antagonist)	25mg Once daily	Mar 2023 – Present	2y 9mo	Minimal improvement in symptoms	CURRENT
Furosemide (Loop Diuretic)	40mg Once daily	Mar 2023 – Present	2y 9mo	Adequate fluid control; edema managed	CURRENT
Sacubitril/ Valsartan (ARNI)	24/26mg Twice daily	Sep 2023 – Nov 2023	2 months	Discontinued due to symptomatic hypotension and dizziness	DISCONTINUED
Ivabradine	5mg Twice daily	Jan 2024 – Mar 2024	2 months	Discontinued – inadequate heart rate reduction	DISCONTINUED

SECTION 2 - TREATMENT RESPONSE SUMMARY

Despite 3 years of optimal medical therapy with maximum tolerated doses of guideline directed medical therapy (GDMT) including beta blocker, ACE inhibitor, and aldosterone antagonist, patient continues to exhibit:

- Persistent LVEF of 35%
- NYHA Class III symptoms (marked limitation in activity)
- Elevated BNP levels (refer to lab report dated 03/10/2026)
- Reduced functional capacity

SECTION 3 - CURRENT MEDICATION LIST

- **Carvedilol:** 25mg twice daily
- **Lisinopril:** 10mg once daily
- **Spironolactone:** 25mg once daily
- **Furosemide:** 40mg once daily
- **Aspirin:** 81mg once daily
- **Atorvastatin:** 40mg once daily

Physician Attestation:

I certify that the above medication history is accurate and complete. The patient has received optimal guideline directed medical therapy for a minimum of 3 years with inadequate clinical response. Advanced cardiac imaging (Cardiac MRI) is required to guide further treatment decisions, including viability assessment and potential ICD placement.

Dr. Ramesh Kumar MD

Board Certified Cardiologist | NPI: 1234567890

Date: March 16, 2026